June 16, 2025

The Honorable Dr. Mehmet Oz
Administrator
Centers for Medicare & Medicaid Services
U.S. Department of Health and Human Services
Attention: CMS-0042-NC
P.O. Box 8013
Baltimore, MD 21244-8013

Submitted electronically to https://www.regulations.gov

RE: CMS-0042-NC Request for Information; Health Technology Ecosystem

Dear Administrator Oz,

On behalf of the Schizophrenia Policy Action Network (SPAN), thank you for the opportunity to submit comments on the CMS Request for Information regarding the health technology ecosystem. SPAN serves as the voice of the schizophrenia community, working to break down the barriers that block people living with schizophrenia from the treatment and support they need to survive – and thrive. We are particularly encouraged by CMS's emphasis on caregiver needs in this RFI, and we offer our perspective grounded in the lived realities of caregivers navigating complex care, including behavioral health systems.

Caregivers of people living with schizophrenia serve as the de facto care coordinators, case managers, and crisis responders for their loved ones. In recent research published by the Schizophrenia and Psychosis Action Alliance (S&PAA), caregivers report lost wages, reduced work hours, and leave of absence from work, resulting in an estimated $104.5 billion in caregiver burden and unpaid labor.[1] Yet, they are often excluded from patient portals, provider communications, and digital navigation tools.

To compound these challenges, schizophrenia is a complex disease to understand and treat. For example, one common symptom of schizophrenia (called anosognosia) is the lack of ability to realize you have the disease, often leading people with schizophrenia to refuse or discontinue treatment. As a result of the treatment interruption, additional burden is shifted to the caregiver to find ways to help – often with few resources and restricted access to information.

Given the complexity of schizophrenia and serious mental illness, we urge CMS to consider the unique circumstances of this caregiver population, and to ensure policies promulgated by the agency reflect the specific needs of caregivers and patients of those with serious mental illness (SMI).

[1] Schizophrenia & Psychosis Action Alliance. (2021, July). *Societal costs of schizophrenia and related disorders* [White paper]. https://sczaction.org/wp-content/uploads/2021/11/571-012_WhitePaper_Report_FINAL_updated_11.09.21.pdf

PC-1. What health management or care navigation apps would help you understand and manage your (or your loved ones) health needs, as well as the actions you should take?

a. **What are the top things you would like to be able to do for you or your loved ones' health that can be enabled by digital health products?**

CMS should support the development and certification of digital health products that provide caregivers (with appropriate consent or proxy status) access to real-time health records, medication history, and appointment scheduling, while enabling alerts for missed prescriptions, hospitalizations, or other indicators of behavioral health crises.

Prescription drug therapeutics (PDTs) also hold promise for enhanced access and quality of care as part of patients' treatment plans. SPAN is encouraged by, and supportive of, advances in PDTs, particularly those that focus on mental health needs.

PC-2. Do you have easy access to your own and all your loved ones' health information in one location (for example, in a single patient portal or another software system)?

c. **Were there particular data types, such as x-rays or specific test results, that were unavailable? What are the obstacles to accessing your own or your loved ones' complete health information electronically and using it for managing health conditions or finding the best care (for example, limitations in functionality, user friendliness, or access to basic technology infrastructure)?**

Interpreting test results, treatment steps, and medication instructions can be especially difficult for caregivers and patients navigating schizophrenia, particularly when English is not their first language. Digital tools that translate medical jargon into plain language—and offer multilingual support—are essential to improving comprehension, adherence, and engagement in care.

PC-5. What can CMS and its partners do to encourage patient and caregiver interest in these digital health products?

a. **What role, if any, should CMS have in reviewing or approving digital health products on the basis of their efficacy, quality or impact or both on health outcomes (not approving in the sense of a coverage determination)? What criteria should be used if there is a review process? What technology solutions, policy changes, or program design changes can increase patient and caregiver adoption of digital health products (for example, enhancements to data access, reimbursement adjustments, or new beneficiary communications)?**

CMS should set caregiver-inclusive standards for usability and functionality. One recommendation is to incorporate a standard caregiver identification question into Medicare and Medicaid annual visits and related digital tools (e.g., patient-facing check-in apps). This simple addition could serve as a routine exercise to identify caregivers early, connect them to appropriate resources, and generate critical data on the scale and nature of caregiving within the Medicare and Medicaid populations. Programs designed with pilots that measure caregiver health, economic stability, and quality of life outcomes as a core part of efficacy could generate certification criteria for digital products that account for caregiver burden reduction.

PC-6. What features are most important to make digital health products accessible and easy to use for Medicare beneficiaries and caregivers, particularly those with limited prior experience using digital tools and services?

To ensure digital health tools are accessible and truly usable by Medicare beneficiaries living with serious mental illness—as well as their caregivers—it is critical to reduce barriers, foster trust, and design systems that accommodate behavioral health complexity.

We recommend, at a minimum, to require features that:

- Use plain, accessible language across all content—especially medication instructions, appointment prompts, and test results. Avoid medical jargon unless clearly explained.
- Include multilingual content to reach caregivers and beneficiaries who speak languages other than English.
- Ensure proxy access features are easy to use and allow caregivers to manage care with appropriate safeguards, especially when the beneficiary cannot consent to treatment changes due to symptoms like anosognosia.

PC-9. Given that the Blue Button 2.0 API only includes basic patient demographic, Medicare coverage, and claims data (Part A, B, D), what additional CMS data sources do developers view as most valuable for inclusion in the API to enable more useful digital products for patients and caretakers. (parts b. & c.)

Digital tools should allow patients to formally designate a caregiver. This simple step improves care coordination, ensures caregivers are recognized in treatment planning, and helps systems direct support to the people keeping care on track after hospitalizations or crises.

PC-12. What are the most valuable operational health data use cases for patients and caregivers that, if addressed, would create more efficient care navigation or eliminate barriers to competition among providers or both?

Real-time access to operational data is especially valuable for caregivers of people with schizophrenia. Being able to see provider availability, contact information, hospitalization status, and medication data is crucial.

Thank you for your leadership and commitment to building a system that works for all. We welcome continued dialogue and would be honored to support CMS's ongoing efforts to develop more inclusive, coordinated digital health policy.

Sincerely,

Gordon Lavigne
CEO
Schizophrenia Policy Action Network (SPAN)